LABORATORY REPORT

Redcliffe labs

Patient NAME	**: Ms SANJYOTI JAGDALE**		
DOB/Age/Gender	: 22 Y/Female	Report STATUS	: Final Report
Patient ID / UHID	: 12354374/RCL11672014	Barcode NO	: 26181365
Referred BY	: Self	Sample Type	: Whole blood EDTA
Sample Collected	: May 08, 2025, 08:47 AM	Report Date	: May 08, 2025, 03:08 PM.

Visit Health - VH00DLT FEMALE with PPBS

Complete Blood Count (CBC)

Test Description	Value(s)	Unit(s)	Reference Range
RBC Parameters			
Hemoglobin *Cyanide free colorimetric*	14.3	g/dL	12.0 - 15.0
RBC Count *Electrical impedance*	4.7	10^6/µl	3.8 - 4.8
PCV *Calculated*	42.4	%	36 - 46
MCV *Calculated*	89.4	fl	83 - 101
MCH *Calculated*	30.2	pg	27 - 32
MCHC *Calculated*	33.8	g/dL	31.5 - 34.5
RDW (CV) *Calculated*	11.8	%	11.6 - 14.0
RDW-SD *Calculated*	41.4	fl	35.1 - 43.9
WBC Parameters			
TLC *Electrical impedance and microscopy*	5.8	10^3/µl	4 - 10
Differential Leucocyte Count			
Neutrophils *Laser based Flow-cytometry*	51.9	%	40-80
Lymphocytes *Laser based Flow-cytometry*	39.4	%	20-40
Monocytes *Laser based Flow-cytometry*	6.2	%	2-10
Eosinophils *Laser based Flow-cytometry*	2.4	%	1-6
Basophils *Laser based Flow-cytometry*	0.1	%	<2
Absolute Leukocyte Counts			
Neutrophils. *Calculated*	3.01	10^3/µl	2 - 7
Lymphocytes. *Calculated*	2.29	10^3/µl	1 - 3
Monocytes. *Calculated*	0.36	10^3/µl	0.2 - 1.0
Eosinophils. *Calculated*	0.14	10^3/µl	0.02 - 0.5
Basophils.	**0.01**	10^3/µl	0.02 - 0.5

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Test Description	Value(s)	Unit(s)	Reference Range
Calculated			
Platelet Parameters			
Platelet Count *Electrical impedance and microscopy*	336	10^3/µl	150 - 410
Mean Platelet Volume (MPV) *Calculated*	**8.8**	fL	9.3 - 12.1
PCT *Calculated*	0.3	%	0.17 - 0.32
P-LCR *Calculated*	33.6	%	18 - 50
P-LCC *Calculated*	113	10^9/L	44 - 140
Mentzer Index *Calculated*	19.02	%	> 13

Interpretation:
CBC provides information about red cells, white cells and platelets. Results are useful in the diagnosis of anemia, infections, leukemias, clotting disorders and many other medical conditions.

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Test Description	Value(s)	Unit(s)	Reference Range

HbA1C (Glycosylated Haemoglobin)

Glycosylated Hemoglobin (HbA1c) *HPLC*	5.1	%	<5.7
Estimated Average Glucose	99.67	mg/dl	Refer Table Below

Interpretation:

Interpretation For HbA1c% As per American Diabetes Association (ADA)

Reference Group	HbA1c in %
Non diabetic adults >=18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5
Therapeutic goals for glycemic control	Age > 19 years Goal of therapy: < 7.0 Age < 19 years Goal of therapy: <7.5

Note:

1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled. 2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate

Comments :

HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations ADA criteria for correlation between HbA1c & Mean plasma glucose levels.

HbA1c(%)	Mean Plasma Glucose (mg/dL)	HbA1c(%)	Mean Plasma Glucose (mg/dL)
6	126	12	298
8	183	14	355
10	240	16	413

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Patient ID / UHID	: 12354374/RCL11672014	Barcode NO	: 26823438
Referred BY	: Self	Sample Type	: Spot Urine
Sample Collected	: May 08, 2025, 08:48 AM	Report Date	: May 08, 2025, 03:08 PM.

Urine Routine and Microscopic Examination

Test Description	Value(s)	Unit(s)	Reference Range
Physical Examination			
Volume	20	mL	-
Colour	Pale yellow	-	Pale yellow
Transparency	Clear	-	Clear
Deposit	Absent	-	Absent
Chemical Examination			
Reaction (pH) *Double Indicator*	6	-	4.5 - 8.0
Specific Gravity *Ion Exchange*	1.010	-	1.010 - 1.030
Urine Glucose (sugar) *Oxidase / Peroxidase*	Negative	-	Negative
Urine Protein (Albumin) *Acid / Base Colour Excahnge*	Negative	-	Negative
Urine Ketones (Acetone) *Legals Test*	Negative	-	Negative
Blood *Peroxidase Hemoglobin*	Negative	-	Negative
Leucocyte esterase *Enzymatic Reaction*	Negative	-	Negative
Bilirubin Urine *Coupling Reaction*	Negative	-	Negative
Nitrite *Griless Test*	Negative	-	Negative
Urobilinogen *Ehrlichs Test*	Normal	-	Normal
Microscopic Examination			
Pus Cells (WBCs)	1-2	/hpf	0 - 5
Epithelial Cells	1-2	/hpf	0 - 4
Red blood Cells	Absent	/hpf	Absent
Crystals	Absent	-	Absent
Cast	Absent	-	Absent
Yeast Cells	Absent	-	Absent
Amorphous deposits	Absent	-	Absent
Bacteria	Absent	-	Absent
Protozoa	Absent	-	Absent

Interpretation:

URINALYSIS- Routine urine analysis assists in screening and diagnosis of various metabolic, urological, kidney and liver disorders.

Protein: Elevated proteins can be an early sign of kidney disease. Urinary protein excretion can also be temporarily elevated by strenuous exercise, orthostatic proteinuria, dehydration, urinary tract infections and acute illness with fever

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Glucose: Uncontrolled diabetes mellitus can lead to presence of glucose in urine. Other causes include pregnancy, hormonal disturbances, liver disease and certain medications.

Ketones: Uncontrolled diabetes mellitus can lead to presence of ketones in urine. Ketones can also be seen in starvation, frequent vomiting, pregnancy and strenuous exercise.

Blood: Occult blood can occur in urine as intact erythrocytes or haemoglobin, which can occur in various urological, nephrological and bleeding disorders.

Leukocytes: An increase in leukocytes is an indication of inflammation in urinary tract or kidneys. Most common cause is bacterial urinary tract infection.

Nitrite: Many bacteria give positive results when their number is high. Nitrite concentration during infection increases with length of time the urine specimen is retained in bladder prior to collection.

pH: The kidneys play an important role in maintaining acid base balance of the body. Conditions of the body producing acidosis/ alkalosis or ingestion of certain type of food can affect the pH of urine.

Specific gravity: Specific gravity gives an indication of how concentrated the urine is. Increased specific gravity is seen in conditions like dehydration, glycosuria and proteinuria while decreased specific gravity is seen in excessive fluid intake, renal failure and diabetes insipidus.

Bilirubin: In certain liver diseases such as biliary obstruction or hepatitis, bilirubin gets excreted in urine.

Urobilinogen: Positive results are seen in liver diseases like hepatitis and cirrhosis and in cases of haemolytic anaemia.

*** End Of Report ***

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